



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رابعياً : رضا مبروك ابورواش الشحات

العنوان : ش النصر

المحافظة : الجيزة

تاريخ تسجيل الإستمارة : 24-09-2026

رقم البطاقة : 26111162100224 رقم التليفون : 01114683007

العمر (تاريخ الميلاد) : 16-11-1961 المهنة : ربه منزل

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإحصائي الاجتماعي (.....) التوقيع: (.....)

المقرر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام ماموجرام وسونار علي الثديين [10888] قسم العلاج : الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : التوقيع :
الاسم : د/ندى حسن الوظيفة : التوقيع :
الاسم : د/نرمين رمضان الوظيفة : التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقر اللجنة بان البيانات المدونة في هذا النموذج صحيحة.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د.ا. / د.ا. / د.ا. / د.ا. /

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

Name: رضا مبروك ابورواش الشحات
National ID: 26111162100224



تقرير طبي

المريض

تحية طبية وبعد,,

بشأن تقرير طبي للمريض

إهداء للطلب المقدم من

رقم طبي:

قسم:

تنتهز هذه بان توضح لهما يلي تقريراً عن حالته

تاريخ ومساءة الخروج :
PM 01:17 05-07-2026

تاريخ ومساءة التحويل :
--- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد انه

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Bilateral mammogram and breast ultrasound

الوقت :

التاريخ :

05-07-2026

الطبيب المعالج :
د. الشيماء لاشين

جمهورية مصر العربية
 بطاقة تخليق الشخصية

رضا
 مبروك ابورواش الشحات
 ١ سن البلوغ من المدة
 مركز أوسيم الجيزة

٢٠١٢/١١/٢٤ ٢٠١٢/١١/٢٤

KH4559460

٢٠١٢/١١ ٢٠١٢/١١/٢٤

ربة منزل

أنثى مسلمة متزوجة
 السيد عمده حسن عبدالرحمن ربابه
 البطاقة مصرية حتى ٢٠٣٠/١١/٢٤

١

Patient Name	رضا مبروك ابورواش	Age	63
Sex	F	Special habits	
Family History	-ve	Comorbidity	DM, HTN, Hysterectomy for fibroids
Surgical History		Telephone	01275407999
Menopausal Status	Post	Marital Status	
ID	26111162100224	Medical ID	1884/25
Date	09-10-2025	Reason of MDT	Post op

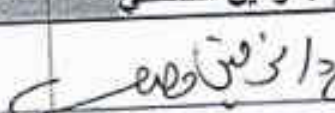
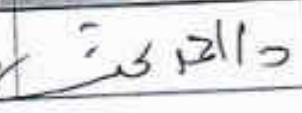
Medical Report

Pathology: Invasive mucinous carcinoma (grade one)
ER: 8/8, PR: 8/8, Her2: +1, Ki67: Requested

S.M: Upfront Rt breast WLE, free margins and ALND (0/10)
T2 N0

Recommendations : ER8/8, PR8/8, Ki67 is DONE

MDT DECISION For: AI (AI FEMARA)

Name	د/ نرمن مصطفى	د/ احمد نجيب	د/ ماهي مختار
Signature			

Patient Name : الأستاذ / رضا مبروك ابوروش	Request No. : 13006150
Patient No. : 26111162100224	Request Date : 01/09/2025
Path No. : 2705/25	Verification Date : 11/09/2025
Sex : Female	Financial Category : جوستاف روسي - نفقة الدولة
Age : 63 Y	Department : Out Patient
Referring Doc :	Registered by : Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Right breast cancer, Up-Front.

Nature Specimen: Right wide local excision & Axillary lymph nodes dissection.

Gross Examination:

- * Right wide local excision specimen measured 10x08x04 cm, with covering skin measured 4x2.5 cm.
- * Serialing showed a glistening mass 3x2.5x1.5 cm, located 2 cm from medial margin, 2.5 cm from lateral margin, 1.5 cm from superior margin, 1 cm from inferior margin & 0.7 cm from the least deep margin.
- * Axillary lymph nodes: Fibrofatty tissue measured 12x8x2 cm, entangling 10 lymph nodes, the largest was 2x1x0.5 cm.

12 Blocks:

- 1- Superior margin., 2- Inferior margin., 3- Lateral margin., 4- Medial margin., 5- Deep margin., 6:8- Mass., 9:12- L.Ns.
- Gross was done by; Dr. Heba Hassan.

Microscopic Examination:

- * Histological Type: Invasive mucinous carcinoma.
- * Grade 1 (differentiation 1, nuclear pleomorphism 2, mitosis 1).
- * 5% Low grade ductal carcinoma in situ (Solid pattern).
- * Angiolinvasion: Absent.
- * Perineural invasion: Absent.
- * Surgical Margins: Free.
- * Axillary lymph nodes: Ten lymph nodes free of metastatic deposits (0/10).

Diagnosis:

Right breast cancer (Up-front), Right wide local excision & Axillary lymph nodes dissection:

Verified by :

Dr.Duaa Mohamed



Patient Name	: الأسنانه / رضا مبروك ابورواش	Request No.	: 13006150
Patient No.	: 26111162100224	Request Date	: 01/09/2025
Path No.	: 2705/25	Verification Date	: 11/09/2025
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 63 Y	Department	: Out Patient
Referring Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

- Invasive mucinous carcinoma, Grade 1.
- 5% Low grade ductal carcinoma in situ (Solid pattern).
- Surgical Margins: Free.
- Axillary lymph nodes: Ten free lymph nodes, (0/10).
- Stage: pT2, pN0(sln).

Signature

Dr. Walaa Ibrahim Ghoneim.

Dr. Heba El-Shafeey.

Verified by :

Dr.Duaa Mohamed

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt



Patient Name : الأستاذ / رضا مبروك ابورواش	Request No. : 13006150
Patient No. : 26111162100224	Request Date : 01/09/2025
Path No. : 2705/25	Verification Date : 05/10/2025
Sex : Female	Financial Category : جوستاف روسي - نفقة الدولة
Age : 63 Y	Department : Out Patient
Referring Doc. :	IHC No. :
	Registered by : Mrs.Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Right breast cancer (Up-front), diagnosed as invasive mucinous carcinoma grade 1.
Nature of Specimen: Right wide local excision & axillary lymph nodes dissection.

Technique:

Sections of the unstained slides, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:

Her2 clone (4B5).

IPT (Positive control showed appropriate reactivity):

Her2: Faint incomplete membrane reaction in more than 10% of tumor cells (score 1+).

Diagnosis:

Right breast cancer (Up-front), Right wide local excision & axillary lymph nodes dissection:

PT:

Her2: Low (score 1+).

Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of

the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

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Room No. (4) - First Floor-Building No, 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name الأستاذ / رضا مبروك ابورواش
Patient No. 26111162100224
Path No. 2705/25
Sex Female
Age 63 Y
Referring Doc.

Request No. 13006150
Request Date 01/09/2025
Verification Date 05/10/2025
Financial Category جوستاف روسي - نفقة الدولة
Department Out Patient

Registered by Mrs. Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

Signature:
Dr. Walaa Ibrahim Ghoneim.
Dr. Duaa Mohamed.

Verified by :

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Room No. (4) - First Floor-Building No, 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient ID: 84265	Patient Name English: Reda Mabrok Abo Rawash
Age: ٦٣ سنة ٧ شهر	2025/06/18 09:32:34 AM Report Date

63/0

Women's Health Outreach Program
Bilateral Full field Digital Mammography

Clinical summary: 65-year-old, right breast lump

Technique

Medio-lateral oblique and CC views High resolution ultrasound

Detailed Findings

Right breast

- Scattered fibro glandular densities, (ACR B).
- UIQ posterior third irregular dense mass
- No distortion or suspicious clusters of micro-calcifications.
- UOQ small nonspecific IMN
- Ultrasound demonstrated a corresponding iso to hypoechoic solid mass with tiny cystic spaces and mild internal vascularity depicted by color Doppler study, it is seen at the 12:00-1:00 outer zone about 0.9cm from the skin and 7 cm from the nipple, measuring 3.8x2.6.5cm
- Prominent retroareolar ducts with few tiny cysts
- Normal skin/subcutaneous tissue and nipple/areolar complex.
- Non-specific axillary LNs with preserved hilum

Left breast

- Scattered fibro glandular densities, (ACR B).
- No masses, asymmetric densities, distortion or suspicious clusters of micro calcifications.
- Ultrasound demonstrated no suspicious solid or cystic lesions
- Prominent retroareolar ducts with few tiny cysts
- Normal skin/subcutaneous tissue and nipple/areolar complex.
- Non-specific axillary LNs.

Impression:

- Right breast: (BIRADS 4c): UIQ palpable suspicious solid mass
- Left breast: (BIRADS 2): Prominent retroareolar ducts with few tiny cysts

Recommendations:

Biopsy was done for histopathological assessment

Dr. Eman Ali

M → DM,
 Hin
 S → Subtotal
 Hypoechoic
 -ve FH

Patient Name: Reda Mabrouk Abo Rawwash Al Shahat	Date : 19/07/2025
Exam : CT CAP(CT Chest Abd. Pelvis)	Age : 63 Year(s)
Medical ID: 1884/2025	Pat ID: 110608

NON CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

**** Giving history of right breast cancer with mastectomy.**

I Chest:-

- Right breast mass measures 3.7 x2.7 cm.
- Bilateral few subcentemetric axillary L. Ns
- Azygous fissure with azygous lobe .
- Few millimetric pulmonary nodules are seen scattered at both lung lobes likely chronic infection.
- Small prevascular enlarged L. N measures 6mm.
- No evidence of pleural effusion, pleural thickening or masses seen.
- No gross cardiac abnormality. No pericardial effusion detected.
- Normal appearance of the chest wall.

II Abdomen & pelvis:

- The liver is of average size with homogenous CT texture. Single hypodense FL seen at segment VII measures 3mm likely cyst.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Distended gall bladder 12.5 x5.5 cm with multiple hyperdense stones and mud.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Normal appearance of the other visualized pelvic organs.
- No abdominal or pelvic masses or collections.

III Scanned skeleton:

- L3, L4 ,L5 and S1 spinolaminectomy with transpedicular fixation with related artifact degrading quality of examination.

Patient Name: Reda Mabrouk Abo Rawwash Al Shahat	Date : 19/07/2025
Exam : CT CAP(CT Chest Abd. Pelvis)	Age : 63 Year(s)
Medical ID: 1884/2025	Pat. ID: 110608

- No definite worrisome lytic or sclerotic focal lesions.

Opinion:

- Right breast mass .
- Small segment VII hypodense FL likely cystic for triphasic MRI.
- No evidence osseous metastatic deposits.
- Calcular mucocele.
-

DR/Reham Khedr,,,MD

Not Approved

Name: Reda Mabrouk

Date: 2025-08-01

PELVI-ABDOMINAL ULTRASONOGRAPHY REVEALS

- **LIVER** is increased in size with bright homogenous echo-pattern. Its contour is regular. No focal lesions or intra-hepatic biliary radicals dilatation.
- Normal **PORTAL VEIN & COMMON BILE DUCT** diameters.
- The midline is concealed with gases.
- **GALL BLADDER** is seen full of clear bile. There are multiple stones , yet no abnormal wall thickening or focal masses.
- **SPLEEN** is average in size with uniform echo-texture. No focal masses.
Normal **SPLENIC VEIN** diameter.
- **RT KIDNEY** shows normal size (9.4 x 5.1 x 2 cm), normal shape & echogenicity, There are renal gravels , yet no backpressure changes or abnormal masses
- **LT KIDNEY** shows normal size (9.5 x 5.4 x 1.5 cm), normal shape & echogenicity, There are renal gravels , yet no backpressure changes or abnormal masses.
- **URINARY BLADDER** is distended with clear urine . no wall thickening or masses or stones .
- **Uterus** is surgically removed.
- No ascites or pelvic collection.
- Colonic gaseous distension.

IMPRESSION

- Bright hepatomegaly ?? fatty.
- Calcular gall bladder.
- Bilateral renal gravels .
- Colonic gaseous distension.

Thanks for reference,
DR/Asmaa.M.Ali

ECHOCARDIOGRAPHY REPORT

Name : REDA MABROUK ABORAWASH
64YRS

Date : 21 August, 2025

Referred by:

Comment:

- Normal L.V internal dimensions with good L.V systolic function.
 - Absent RWMA (IHD can't be excluded at rest)
 - Diastolic dysfunction grade I (impaired relaxation pattern
- Normal RV size and systolic function, mild TR, estimated
PAF ~ 28 mmhg
- No masses or thrombi.
 - Normal pericardium
 - Intact interventricular septum.

DR. PETER KAMAL,
MD Cardiology



ECHOCARDIOGRAPHY REPORT

Name : REDA MABROUK ABORAWASH
64YRS

Date : 21 August, 2025

Referred by:

Comment:

- Normal L.V internal dimensions with good L.V systolic function.
- Absent RWM/A (IHD can be excluded at rest
- Diastolic dysfunction grade I (impaired relaxation pattern
- Normal RV size and systolic function, mild TR , estimated PAP~ 28 mmHg
- No masses or thrombi.
- Normal pericardium
- Intact interventricular septum.

DR. PETER KAMAL,
MD Cardiology



Patient ID: 84255	Patient Name English: Ruds Mabrok Abo Rawash
2025/06/18 09:32:34 AM	Report Date
Page 1 of 1	

63/0

$M \rightarrow DM$,
 $Hin \rightarrow Subtotal$
 $S \rightarrow Hybrid$
 $-ve FH$

Women's Health Outreach Program
Bilateral Full field Digital Mammography

Clinical summary:	65-year-old, right breast lump	High resolution ultrasound
Technique	Ultrasound and CC views	

Technique

Medio-lateral oblique and CC views

Detailed Findings

Right breast

- Scattered fibro glandular densities, (ACR B).
 - UIQ posterior third irregular dense mass
 - No distortion or suspicious clusters of micro-calcifications.
 - UOQ small nonspecific IMN
 - Ultrasound demonstrated a corresponding iso to hypoechoic solid mass with tiny cystic spaces and mild internal vascularity depicted by color Doppler study, it is seen at the 12:00-1:00 outer zone about 0.9cm from the skin and 7 cm from the nipple, measuring 3.8x2.6.5cm
 - Prominent retroareolar ducts with few tiny cysts
 - Normal skin/subcutaneous tissue and nipple/areolar complex.
 - Non-specific axillary LNs with preserved hilum
- Left breast
- Scattered fibro glandular densities, (ACR B).
 - No masses, asymmetric densities, distortion or suspicious clusters of micro calcifications.
 - Ultrasound demonstrated no suspicious solid or cystic lesions
 - Prominent retroareolar ducts with few tiny cysts
 - Normal skin/subcutaneous tissue and nipple/areolar complex
 - Non-specific axillary LNs.

Impression:

- Right breast: (BIRADS 4c): UIQ palpable suspicious solid mass
- Left breast: (BIRADS 2): Prominent retroareolar ducts with few tiny cysts

Recommendations:

commentary
Biopsy was done for histopathological assessment.

Dr. Eman Ali

E/ V/ A

Patient Name	الأمستاف / رضا مبروك ابوروش :	Request No.	: 13006150
Patient No.	: 26111162100224	Request Date	: 01/09/2025
Path No.	: 2705/25	Verification Date	: 11/09/2025
Sex	: Female	Financial Category	: نفقة الدولة - جوستاف روسي
Age	: 63 Y	Department	: Out Patient
Referreing Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Right breast cancer, Up-Front.

Nature Specimen: Right wide local excision & Axillary lymph nodes dissection.

Gross Examination:

- Right wide local excision specimen measured 10x08x04 cm, with covering skin measured 4x2.5 cm.
- Serialing showed a glistening mass 3x2.5x1.5 cm, located 2 cm from medial margin, 2.5 cm from lateral margin, 1.5 cm from superior margin, 1 cm from inferior margin & 0.7 cm from the least deep margin.
- Axillary lymph nodes: Fibrofatty tissue measured 12x8x2 cm, entangling 10 lymph nodes, the largest was 2x1x0.5 cm.

12 Blocks:

- 1- Superior margin., 2- Inferior margin., 3- Lateral margin., 4- Medial margin., 5- Deep margin., 6:8- Mass., 9:12- L.Ns.
- Gross was done by; Dr. Heba Hassan.

Microscopic Examination:

- Histological Type: Invasive mucinous carcinoma.
- Grade 1 (differentiation 1, nuclear pleomorphism 2, mitosis 1).
- 5% Low grade ductal carcinoma in situ (Solid pattern).
- Angiolinvasion: Absent.
- Perineural invasion: Absent.
- Surgical Margins: Free.
- Axillary lymph nodes: Ten lymph nodes free of metastatic deposits (0/10).

Diagnosis:

Right breast cancer (Up-front), Right wide local excision & Axillary lymph nodes dissection:

Verified by :

Dr.Duaa Mohamed

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Page 1 of 2

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Room No. (4) - First Floor-Building No. 987- Nile Corniche

AVE/
SSY
A

Patient Name : الأستاذ / رضا مبروك لورواش
Patient No. : 26111162100224
Path No. : 2705/25
Sex : Female
Age : 63 Y
Referreing Doc :

Request No. : 13006150
Request Date : 01/09/2025
Verification Date : 11/09/2025
Financial Category : نفقة الدولة - جوماتف روسي
Department : Out Patient
Registered by : Mrs.Mayada Hegazy

Pathology Report

- Invasive mucinous carcinoma, Grade 1.
- 5% Low grade ductal carcinoma in situ (Solid pattern).
- Surgical Margins: Free.
- Axillary lymph nodes: Ten free lymph nodes, (0/10).
- Stage: pT2, pN0(sln).

Signature

Dr. Walaa Ibrahim Ghoneim.
Dr. Heba El-Shafeey.

Verified by :
Dr.Duaa Mohamed

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Page 2 of 2

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name	رضا مبروك ابورواش	Age	63
Sex	F	Special habits	
Family History	-ve	Comorbidity	DM, HTN, Hysterectomy for fibroids
Surgical History		Telephone	01275407999
Menopausal Status	Post	Marital Status	
ID	26111162100224	Medical ID	1884/25
Date	09-10-2025	Reason of MDT	Post op

Medical Report

Pathology: Invasive mucinous carcinoma (grade one)
ER: 8/8, PR: 8/8, Her2: +1, Ki67: Requested

S.M: Upfront Rt breast WLE, free margins and ALND (0/10)
T2 N0

Recommendations : ER8/8, PR8/8, Ki67 is DONE

MDT DECISION For: AI (AI FEMARA)

PORT

Name	د/ نرمن مصطفى	د/ احمد نجيب	د/ ماهي مختار
Signature			



١٢٠٧٨ ١١٧٧

١٢٧٥٣-٧٩٩٩

١٢



٢٠٢٣/١١

٢٣١٥١١٦٦١٠٠٢٢٤

ربة منزل

متزوجة

مستعدة

التي

السيد عياد حسن عواد حسن زكية

البلقة صارية حتى ٢٠٢٠/١١/٢١





معامل المدينة لـب للتحاليل الطبية

Request Date	02/08/2025	Lab.ID	36533
Sex	Male	Age	64 Years
Reporting Date	03/08/2025	Name	رضا ميرزاك ابو رواش
		Referred By	أ.د. / ياسر المسوري

Urine analysis

Physical Examination:

Volume	Sample
Colour	Yellow
Aspect	Turbid
Sp. Gr	1020
Reaction (pH)	Acidic

Chemical Examination:

Albumin	Nil
Glucose	+++
ketones	Nil
Bilirubin	Nil
Urobilinogen	Normal Trace

Microscopic Examination

Pus cells	20-25*	/HPF
RBCs	3-5	/HPF
Epithelial Cells	++	
Casts	Nil	
Crystals	Nil	
Amorphous	Urate (++)	
Parasites	Nil	
Others	Nil	

Signature

م.م. أطيب التمهيات بالمشفء العام

01148070325

01129237826

ECHOCARDIOGRAPHY REPORT

Name : REDA MABROUK ABORAWASH
64YRS

Date : 21 August, 2025

Referred by:

Comment:

- Normal L.V internal dimensions with good L.V systolic function.
 - Absent RWMA (IHD can't be excluded at rest)
 - Diastolic dysfunction grade I (impaired relaxation pattern
- Normal RV size and systolic function, mild TR, estimated
PAF~ 28 mmhg
- No masses or thrombi.
 - Normal pericardium
 - Intact interventricular septum.

DR. PETER KAMAL,
MD Cardiology



ECHOCARDIOGRAPHY REPORT

Name : REDA MABROUK ABORAWASH
64YRS

Date :21 August, 2025

Referred by:

Comment:

- Normal L.V internal dimensions with good L.V systolic function.
- Absent RWM/A (IHD can be excluded at rest
- Diastolic dysfunction grade I(impaired relaxation pattern
- Normal RV size and systolic function, mild TR , estimated PAP~ 28 mmHg
- No masses or thrombi.
- Normal pericardium
- Intact interventricular septum.

DR. PETER KAMAL,
MD Cardiology



Patient Name : رضا مبروك ابو رزائى
Patient No : 1884-2025
Sex : Female Age : 64 Y
Financial Cate : نقية الاولى - جويستف روسي
Department : Out Patient
Referring Doc :

Lab No. : 13001528
Request Date : 11-08-2025 11:27
Verification Date : 11-08-2025 12:18
Sample Type : EDTA Blood
Test Status : Routine
Analyzers : DXH_600_Hermel

Complete Blood Picture

Test	Result	Unit	Ref. Range
Hemoglobin	11.5	g/dL	12.0 - 16.0
RBCs	4.3	$10^6/\mu\text{L}$	4.0 - 6.5
Haematocrit (PCV)	<u>33.1</u>	%	36.0 - 50.0
MCV	77.3	fL	76.0 - 101.0
MCH	26.8	pg	26.0 - 32.0
MCHC	<u>34.7</u>	g/dL	31.5 - 34.5
RDW-CV	<u>14.7</u>	%	11.6 - 14.0
PLT	416	$10^3/\mu\text{L}$	150 - 450
WBCs.Total	6.5	$10^3/\mu\text{L}$	4.0 - 11.5

Differential Leucocyte Count :

	Relative	Absolute	
Neutrophils%	62.0	4.03	/cmm 2.00 - 7.00
Lymphocytes%	28.5	1.85	/cmm 1.50 - 3.50
Monocytes%	6.7	0.44	/cmm 0.20 - 1.00
Eosinophils%	2.3	0.15	/cmm Up to 0.50
Basophils%	0.5	0.03	/cmm Up to 0.10

Signature

Dr./Manal Mokhtar

Patient Name : **المنيرة / ريماء جورجك الجورجسي**
 Patient No. : 2611162100224
 Path No. : SR-969/25
 Sex : Female
 Age : 63 Y
 Referring Doc :

Request No. : 13006122
 Request Date : 17/07/2025
 Verification Date : 20/07/2025
 Financial Category : فئة المرضى - Out Patient
 Department :
 Registered by : Mrs. Mayada Hegazy

Pathology Report

Clinical Data: Right breast cancer
Nature of Specimen: Referred block & slides (Tru-cut biopsy).

Gross Examination:

One referred block labelled by (121-25). Together with, five referred slides including 1 H&E stained slide and 4 immunostained slides for (ER, PR, HER2neu & Ki67).

Microscopic Examination:

- Histological type: Invasive mucinous carcinoma.
- Grade 1 (differentiation 2, nuclear pleomorphism 2, mitosis 1).
- DCIS: Absent
- Angiolinvasion: Absent
- Perineural invasion: Absent.

• IPT:

- ER: Strong nuclear reaction in 98% of tumor cells (score 8/8).
- PR: Strong nuclear reaction in 98% of tumor cells (score 8/8).
- Her2: Negative membrane reaction in tumor cells (score 0).
- Ki67%: 5%.

Diagnosis:

Right breast mass, Referred block & slides (Tru-cut biopsy):

- Invasive mucinous carcinoma, Grade 1.

IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 8/8).
- Her2: Negative (score 0).
- Ki67%: 5%.

Verified by :

Dr. Duza Mohamed

Printing Date : 28/07/2025 14:23:54

Page 1 of 2

Printing By : Mrs. Mayada Hegazy



Patient Name : الأستاذ / د. رضا مبروك البروراني
Patient No. : 2611162100224
Path No. : SR-969/25
Sex : Female
Age : 63 Y
Referring Doc :

Request No. : 13006122
Request Date : 17/07/2025
Verification Date : 20/07/2025
Financial Category : فئة المرضى - جراحة
Department : Out Patient
Registered by : Mrs. Mayada Hegazy

Pathology Report

* Scoring system: "ER, PR"
ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"
Reference: ASCO-CAP Her2 Test Guideline Recommendations. 2013.IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faintly perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faintly perceptible and within > 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within > 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature

Dr. Walaa Ibrahim Ghoneim.
Dr. Heba Hassan Adly.

Verified by :
Dr. Duaa Mohamed

Printing Date : 28/07/2025 14:13:54

Page 2 of 2

Printing By : Mrs. Mayada Hegazy



Patient Name : الأستاذ / رضا مبروك الوروداني
Patient No. : 2611162100224
Path No. : 2705/25
Sex : Female
Age : 63 Y
Referring Doc :

Pathology Report

Request No. : 13006150
Request Date : 01/09/2025
Verification Date : 11/09/2025
Financial Category : نفقة الوردي :
Department : Out Patient
Registered by : Mrs. Mayada Hegazy

Clinical Data: Right breast cancer, Up-Front.
Nature Specimen: Right wide local excision & Axillary lymph nodes dissection.

Gross Examination:

- Right wide local excision specimen measured 10x08x04 cm, with covering skin measured 4x2.5 cm.
- Serialing showed a glistening mass 3x2.5x1.5 cm, located 2 cm from medial margin, 2.5 cm from lateral margin, 1.5 cm from superior margin, 1 cm from inferior margin & 0.7 cm from the least deep margin.
- Axillary lymph nodes: Fibrofatty tissue measured 12x8x2 cm, entangling 10 lymph nodes, the largest was 2x1x0.5 cm.

12 Blocks:

- 1- Superior margin., 2- Inferior margin., 3- Lateral margin., 4- Medial margin., 5- Deep margin., 6:8- Mass., 9:12- L.Ns.
- Gross was done by: Dr. Heba Hassan.

Microscopic Examination:

- Histological Type: **Invasive mucinous carcinoma.**
- Grade 1 (differentiation 1, nuclear pleomorphism 2, mitosis 1).
- 5% Low grade ductal carcinoma in situ (Solid pattern).
- Angiolinvasion: Absent
- Perineural invasion: Absent.
- Surgical Margins: Free.
- Axillary lymph nodes: Ten lymph nodes free of metastatic deposits (0/10).

Diagnosis:

Right breast cancer (Up-front), Right wide local excision & Axillary lymph nodes dissection:

Verified by :

Printing Date : 09/10/2025 08:18:16

Page 1 of 2

Printing By : thermal



Patient Name : الأستلا / رضا مبروك لوزراني
Patient No. : 26111162100224
Path No. : 2705/25
Sex : Female
Age : 63 Y
Referring Doc. :

Request No. : 13006150
Request Date : 01/09/2025
Verification Date : 05/10/2025
Financial Category : جويستاف روسي - نفقة الدولة
Department : Out Patient
IHC No. :
Registered by : Mrs. Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Right breast cancer (Up-front), diagnosed as invasive mucinous carcinoma grade 1.
Nature of Specimen: Right wide local excision & axillary lymph nodes dissection.

Technique:

Sections of the unstained slides, processed in Bench Mark ULTRA machine (Ventana) and stained by Abbot monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counterstain. The sections treated against:

Her2 clone (4B5).

IPT (Positive control showed appropriate reactivity):

Her2: Faint incomplete membrane reaction in more than 10% of tumor cells (score 1+).

Diagnosis:

Right breast cancer (Up-front), Right wide local excision & axillary lymph nodes dissection.
>T:

Her2: Low (score 1+).

Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

1C 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within > 10% of

the invasive tumor cells or complete and circumferential membrane staining that is intense and within > 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Verified by :

Printing Date : 09/10/2025 08:17:00

Page 1 of 2

Printed By : hemal



Patient Name: Reda Mabrouk Abo Rawwash Al Shahat	Date : 19/07/2025
Exam : CT CAP(CT Chest Abd. Pelvis)	Age : 63 Year(s)
Medical ID: 1884/2025	Pat. ID: 110608

NON CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

**** Giving history of right breast cancer with mastectomy.**

I Chest:-

- Right breast mass measures 3.7 x2.7 cm.
- **Bilateral few subcentimetric axillary L. Ns**
- Azygous fissure with azygous lobe .
- Few millimetric pulmonary nodules are seen scattered at both lung lobes likely chronic infection.
- Small prevascular enlarged L. N measures 6mm.
- No evidence of pleural effusion, pleural thickening or masses seen.
- No gross cardiac abnormality. No pericardial effusion detected.
- Normal appearance of the chest wall.

II Abdomen & pelvis:

- The liver is of average size with homogenous CT texture. Single hypodense FL seen at segment VII measures 3mm likely cyst.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Distended gall bladder 12.5 x5.5 cm with multiple hyperdense stones and mud.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Normal appearance of the other visualized pelvic organs.
- No abdominal or pelvic masses or collections.

III. Scanned skeleton:

- L3, L4 ,L5 and S1 spinolaminectomy with transpedicular fixation with related artifact degrading quality of examination.

Patient Name: Reda Mabrouk Abo Rawwash Al Shahat	Date : 19/07/2025
Exam : CT CAP(CT Chest Abd. Pelvis)	Age : 63 Year(s)
Medical ID: 1884/2025	Pat ID: 110608

- No definite worrisome lytic or sclerotic focal lesions.

Opinion:

- Right breast mass .
- Small segment VII hypodense FL likely cystic for triphasic MRI.
- No evidence osseous metastatic deposits.
- Calcular mucocele.
-

DR/Reham Khedr,,,MD

Not Approved

Patient Name : الأستاذ / رضا مبروك البوراشي
Patient No. : 2611162100224
Path No. : 2705/25
Sex : Female
Age : 63 Y
Refereing Doc. :

Request No. : 13006150
Request Date : 01/09/2025
Verification Date : 05/10/2025
Financial Category : فئة المرضى
Department : Out Patient
IHC No. :
Registered by : Mrs.Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Right breast cancer (Up-front), diagnosed as invasive mucinous carcinoma grade 1.
Nature of Specimen: Right wide local excision & axillary lymph nodes dissection.

technique:

Sections of the unstained slides, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:

Her2 clone (4B5).

IPT (Positive control showed appropriate reactivity):

Her2: Faint incomplete membrane reaction in more than 10% of tumor cells (score 1+).

diagnosis:

Right breast cancer (Up-front), Right wide local excision & axillary lymph nodes dissection.
PT.

Her2: Low (score 1+).

Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013.IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of
the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 0% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Verified by :

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Page 1 of 2

Printed By : hemal





خبرة (24 ساعة)

Name: Reda Mabrouk

Date: 2025-08-01

PELVI-ABDOMINAL ULTRASONOGRAPHY REVEALS

- LIVER** is increased in size with bright homogenous echo-pattern. Its contour is regular. No focal lesions or intra-hepatic biliary radicals dilatation.
- Normal **PORTAL VEIN & COMMON BILE DUCT** diameters.
- The midline is concealed with gases.
- GALL BLADDER** is seen full of clear bile. There are multiple stones . yet no abnormal wall thickening or focal masses.
- SPLEEN** is average in size with uniform echo-texture. No focal masses.
- Normal **SPLENIC VEIN** diameter.
- RT KIDNEY** shows normal size (9.4 x 5.1 x 2 cm), normal shape & echogenicity, There are renal gravels , yet no backpressure changes or abnormal masses
- LT KIDNEY** shows normal size (9.5 x 5.4 x 1.5 cm), normal shape & echogenicity, There are renal gravels , yet no backpressure changes or abnormal masses.
- URINARY BLADDER** is distended with clear urine . no wall thickening or masses or stones .
- Uterus* is surgically removed.
- No ascites or pelvic collection.
- Colonic gaseous distension.

IMPRESSION

- Bright hepatomegaly ?? fatty.
- Calcular gall bladder.
- Bilateral renal gravels .
- Colonic gaseous distension.

Thanks for reference,
DR/Asmaa.M.Ali

Patient ID: 84265	Patient Name English: Rada Mabrouk Abo Rawash
MR V Scan #1 Age:	2025/06/18 (09:32:34 AM) Report Date

63/10

Women's Health Outreach Program

Bilateral Full Field Digital Mammography

Clinical summary: 65-year-old, right breast lump

Technique

Medio-lateral oblique and CC views High resolution ultrasound

Detailed Findings

Right breast

- Scattered fibro glandular densities, (ACR B).
- UIQ posterior third irregular dense mass
- No distortion or suspicious clusters of micro-calcifications.
- UOQ small nonspecific IMN
- Ultrasound demonstrated a corresponding iso to hypoechoic solid mass with tiny cystic spaces and mild internal vascularity depicted by color Doppler study, it is seen at the 12:00-1:00 outer zone about 0.9cm from the skin and 7 cm from the nipple, measuring 3.8x2.6.5cm
- Prominent retroareolar ducts with few tiny cysts
- Normal skin/subcutaneous tissue and nipple/areolar complex.
- Non-specific axillary LNs with preserved hilum

Left breast

- Scattered fibro glandular densities, (ACR B).
- No masses, asymmetric densities, distortion or suspicious clusters of micro calcifications.
- Ultrasound demonstrated no suspicious solid or cystic lesions
- Prominent retroareolar ducts with few tiny cysts
- Normal skin/subcutaneous tissue and nipple/areolar complex.
- Non-specific axillary LNs.

Impression:

- Right breast: (BIRADS 4c): UIQ palpable suspicious solid mass
- Left breast: (BIRADS 2): Prominent retroareolar ducts with few tiny cysts

Recommendations:

Biopsy was done for histopathological assessment

Dr. Eman Ali

Handwritten notes:
 H → DM
 S → Subtotal
 Hydronephrosis
 -ve fH

Patient Name : الأستاذ / رضا مبروك ابوروش
 Patient No. : 26111162100224
 Path No. : 2705/25
 Sex : Female
 Age : 63 Y
 Request No. : 13006150
 Request Date : 01/09/2025
 Verification Date : 11/09/2025
 Financial Category : نقدة الدولة - جوستاف روسي
 Department : Out Patient
 Registered by : Mrs. Mayada Hegazy

Pathology Report

Clinical Data: Right breast cancer, Up-Front.

Nature Specimen: Right wide local excision & Axillary lymph nodes dissection.

Gross Examination:

- Right wide local excision specimen measured 10x08x04 cm, with covering skin measured 4x2.5 cm.
- Serialing showed a glistening mass 3x2.5x1.5 cm, located 2 cm from medial margin, 2.5 cm from lateral margin, 1.5 cm from superior margin, 1 cm from inferior margin & 0.7 cm from the least deep margin.
- Axillary lymph nodes: Fibrofatty tissue measured 12x8x2 cm, entangling 10 lymph nodes, the largest was 2x1x0.5 cm.

12 Blocks:

- 1- Superior margin., 2- Inferior margin., 3- Lateral margin., 4- Medial margin., 5- Deep margin., 6:8- Mass., 9:12- L.Ns.
- Gross was done by; Dr. Heba Hassan.

Microscopic Examination:

- Histological Type: Invasive mucinous carcinoma.
- Grade 1 (differentiation 1, nuclear pleomorphism 2, mitosis 1).
- 5% Low grade ductal carcinoma in situ (Solid pattern).
- Angiolinvasion: Absent.
- Perineural invasion: Absent.
- Surgical Margins: Free.
- Axillary lymph nodes: Ten lymph nodes free of metastatic deposits (0/10).

Diagnosis:

Right breast cancer (Up-front), Right wide local excision & Axillary lymph nodes dissection:

Verified by :

Dr.Duaa Mohamed

Printing Date : 16/09/2025 10:57:22

Page 1 of 2

Printing By : Mrs. Mayada Hegazy



مصر - مصر القديمة - القاهرة - مصر
 Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt
 غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل

Patient Name : الاستاذة / رضا مبروك لورواش
Patient No. : 26111162100224
Path No. : 2705/25
Sex : Female
Age : 63 Y
Referreing Doc :

Request No. : 13006150
Request Date : 01/09/2025
Verification Date : 11/09/2025
Financial Category : نفقة الدولة - جومستاف روسي
Department : Out Patient
Registered by : Mrs.Mayada Hegazy

Pathology Report

- Invasive mucinous carcinoma, Grade 1.
- 5% Low grade ductal carcinoma in situ (Solid pattern).
- Surgical Margins: Free.
- Axillary lymph nodes: Ten free lymph nodes, (0/10).
- Stage: pT2, pN0(sln).

Signature

Dr. Walaa Ibrahim Ghoneim.
Dr. Heba El-Shafeey.

Verified by :

Dr.Duaa Mohamed

Printing Date : 16/09/2025 10:57:22

Page 2 of 2

Printing By : Mrs.Mayada Hegazy



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

التشخيص:

التشخيص:

جميع الأوسر الطبية لإد أن تكون واضحة وموقعة من الطبيب

Patient Name	رضا مبروك ابو رواش	Age	63
Sex	F	Special habits	
Family History	-ve	Comorbidity	DM, HTN, Hysterectomy for fibroids
Surgical History		Telephone	01275407999
Menopausal Status	Post	Marital Status	
ID	26111162100224	Medical ID	1884/25
Date	09-10-2025	Reason of MDT	Post op

Medical Report

Pathology: Invasive mucinous carcinoma (grade one)
ER: 8/8, PR: 8/8, Her2: +1, Ki67: Requested

S.M: Upfront Rt breast WLE, free margins and ALND (0/10)
T2 N0

Recommendations : ER8/8, PR8/8, Ki67 is DONE

MDT DECISION For: AI (AI FEMARA)

POA

Name	د/ نرمن مصطفى	د/ احمد نجيب	د/ ماضي مختار
Signature			



MDT FORM

49 / 40 / 2025 G

Name	رضا مبروك ابو وائل	MRN	1883 / 2
Age	63	ID	57111751.0552
Menopausal status	Post	Phone	01275407999
NRS SCORE		Marital status	-ve
BMI		Family history	postoperative
CO morbidity	DM, HTN	REASON OF MDT	Hysterectomy for Fibroids

Clinical Data

R

L

BX: Invasive mucinous Carcinoma GI

ER: 8/8 PR: 8/8 HER 2: -ve Ki67: 5%

Sonomammography:

No data available in the file.

Mets Workup:

T N M

Recommendations:

upfront RT 33 w/ RT, two
Rt breast w/ RT, two
margins & ALND (0/10)

T2 NO

1/1/25

MDT OUTCOME & PLAN

For ER, PR, Her2, Ki67 +
AT (female) 9/10/20

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
For ER, PR, Her2, Ki67 + AT (female) 9/10/20					